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How to Write a Medical Case Report

A Research Report

A practical guide for medical students, junior doctors, nurses, and allied health professionals who want to turn an interesting patient encounter into a clear, honest, and publishable case report.

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Note: All patient examples in this report are fictional and invented for teaching purposes. No real patient is described.

Abstract

A medical case report is a short, structured account of a single patient, or a small group of patients, whose presentation, diagnosis, treatment, or outcome teaches something useful to other clinicians. Although case reports sit near the bottom of the traditional evidence hierarchy, they remain one of the most important ways in which new diseases are recognised, rare side effects are first noticed, and unusual presentations of common conditions are shared. Many major discoveries in medicine began as a single careful observation written down well.

This report explains, step by step, how to write a case report that other clinicians will actually read, trust, and learn from. It begins by defining what a case report is and when a case is worth reporting, and it introduces the CARE guidelines, the internationally accepted structure for case reports. It then works through each section in detail: the title and abstract, the introduction, the case presentation with history, examination, investigations and a timeline, the treatment and outcome, and the discussion. A full chapter is devoted to patient consent, anonymity, and ethics, because a case report can never be published at the cost of a patient's privacy or dignity.

The report also contains a complete worked example: a clearly labelled fictional case of a patient with an unusual presentation of a common condition, written out in full as a model that a beginner can imitate. Finally, it lists the most common mistakes that lead to rejection by journals and provides a final checklist that authors can use before submission. The aim throughout is practical: by the end of this report, a student or junior clinician should be able to select a suitable case, gather the facts carefully, obtain consent, and write a clear, honest, and well-structured report.

Keywords: case report; CARE guidelines; medical writing; clinical observation; patient consent; medical ethics.

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Chapter 1: Introduction — What a Medical Case Report Is and Why It Matters

A medical case report is a detailed, structured description of one patient (or, occasionally, a very small number of patients) that focuses on something unusual, instructive, or new. Unlike a research paper, which tests a hypothesis in a large group, a case report describes what happened to one person and what clinicians can learn from it. It is the oldest form of medical writing: doctors have been sharing interesting cases with each other for centuries, long before randomised trials existed.

Case reports matter for several practical reasons. First, they are often how new conditions are first recognised. When several doctors independently report similar unusual cases, a pattern emerges, and a new disease or syndrome can be identified. Second, they alert the profession to rare side effects of drugs and treatments. Many drug warnings began with a single case report of an unexpected reaction. Third, they teach clinical reasoning: they show how a diagnosis was reached, what was confusing, what went wrong, and what finally helped. A good case report reads a little like a detective story, and that is why students and busy clinicians enjoy reading them.

Case reports also matter for the author. For a medical student or junior doctor, writing a case report is often the first step into academic publishing. It teaches how to take a careful history, review the literature, organise information logically, write clearly, and respond to reviewers. It does not require a large research team, funding, or ethical approval for a trial. It requires curiosity, careful notes, honesty, and respect for the patient.

What a Case Report Is Not

A case report is not a full review of a disease, and it is not a small research study. It should not try to prove that a treatment works, because one patient cannot prove effectiveness. It should not present a routine case with nothing new to teach. And it must never exaggerate: a single observation is a suggestion, not proof. The strength of a case report lies in careful description and honest interpretation, not in grand claims.

Who Reads Case Reports?

Case reports are read by clinicians who may one day meet a similar patient, by students learning to recognise patterns, by researchers looking for new questions to study, and by editors and reviewers judging whether a case deserves wider attention. A well-written report therefore serves two audiences at once: the busy clinician who wants the lesson in two minutes, and the careful reader who wants enough detail to judge whether the lesson is trustworthy.

Chapter 2: When Is a Case Worth Reporting?

Not every interesting patient should become a case report. Journals receive far more submissions than they can publish, and the first question an editor asks is simple: what will readers learn from this that they do not already know? A case is worth reporting when it meets at least one of the following tests.

- A new or rare disease, or a rare presentation of a common disease. For example, a common infection appearing in an unexpected organ, or a familiar condition occurring at a very unusual age.
- An unexpected association between two conditions, or between a drug and a side effect that has not been described before.
- An unexpected outcome: a patient who recovered when recovery seemed unlikely, or who deteriorated despite standard treatment.
- A new diagnostic clue, a new use of an existing test, or a diagnostic pitfall that could mislead others.
- A new or modified treatment approach, or a useful lesson about managing a difficult situation with limited resources.
- A case that challenges current understanding or guidelines, and suggests that they may need to be reconsidered.

Before investing weeks of work, test the idea honestly. Search the literature: has this exact case already been reported many times? If so, your report adds little unless your case differs in an important way. Ask a senior colleague: would you find this instructive? And ask yourself: can I describe the case completely and accurately from the records? A case with missing notes, lost investigations, or an unclear outcome is very difficult to publish, however interesting it seemed at the bedside.

The 'So What?' Test

Every case report must pass the 'so what?' test. After reading it, the reader should be able to say what they will do differently next time. If the honest answer is 'nothing', the case is probably not worth reporting. Write the lesson in one sentence before you start: 'This case shows that...' If you cannot finish that sentence clearly, keep thinking, or choose a different case.

When Not to Write a Case Report

- The case is routine and the outcome was exactly as expected.
- The records are incomplete and key facts cannot be verified.
- The patient (or their family) does not consent, or cannot be contacted for consent.
- The main purpose is to advertise a clinic, a product, or yourself, rather than to teach.

- You are reporting a case primarily because it is rare, but you have no useful lesson, no clear diagnosis, and no follow-up.

Chapter 3: The CARE Guidelines: The Standard Structure for Case Reports

The CARE guidelines — CARE stands for CAse REport — are the internationally accepted reporting standard for case reports. They were developed by an international group of clinicians, researchers, and journal editors to make case reports more complete, transparent, and useful. Most medical journals now expect authors to follow them, and many ask authors to submit a completed CARE checklist with their manuscript.

The CARE guidelines do not tell you what to think; they tell you what information to include so that readers can judge your case for themselves. Following them protects you against the most common reason for rejection: missing information that the reader needs. The checklist covers the title, keywords, abstract, introduction, patient information, clinical findings, timeline, diagnostic assessment, interventions, follow-up and outcomes, discussion, patient perspective, and informed consent.

The Standard Structure at a Glance

Section	What It Contains
Title	Concise, informative; includes the words 'case report' and the key feature.
Abstract	Brief summary: background, case, outcome, lesson (often structured).
Introduction	Why this case matters; brief background with a few key references.
Case presentation	Patient information, history, examination, investigations, timeline.
Treatment	What was done, doses, duration, and any changes made.
Outcome	What happened, follow-up duration, and the patient's current state.
Discussion	What is new, how it compares with the literature, limitations.
Patient perspective	The patient's own view, when available and appropriate.
Consent	Statement that written informed consent was obtained.

You do not need to memorise the full CARE checklist before your first report, but you should read it once and keep it beside you while writing. It is freely available online

from the CARE guidelines group and from many journal websites. Using it is a sign of professionalism: it tells editors that you take reporting standards seriously.

Why Structure Matters

A standard structure helps readers in a hurry. A clinician searching for a similar case wants to find the timeline quickly, check the investigations, and read the outcome, without wading through pages of background. Structure also helps reviewers: they can check each section in turn and see what is missing. Finally, structure helps you: it stops you from forgetting essential details, such as the duration of follow-up or the exact dose of a drug, which are easy to omit when writing freely.

Chapter 4: Title and Abstract: Getting Found and Read

The title and abstract are the most-read parts of any case report. Most readers will never go further: they will find your report through a search, read the title, read the abstract, and decide in seconds whether to continue. These two small sections therefore deserve more care than their length suggests.

Writing a Good Title

A good title is specific, honest, and searchable. It should name the condition or the key feature of the case, and it should usually include the phrase 'case report' so that databases and readers can identify the article type immediately. Avoid vague titles such as 'An Interesting Case' or 'A Rare Case', which tell the reader nothing. Avoid sensational titles that promise more than the case delivers.

- Good: 'Unusual Presentation of Dengue Fever with Prolonged Hiccups: A Case Report'
- Good: 'Appendicitis Presenting as Right Shoulder Pain in a Young Adult: A Case Report'
- Weak: 'An Interesting Case from Our Hospital' — no condition, no lesson, not searchable.
- Weak: 'A Miraculous Recovery' — sensational, vague, and unprofessional.

Include the most important keywords in the title itself: the disease, the unusual feature, and the population if it matters (for example, 'in a child' or 'in pregnancy'). Think about what words a clinician would type into a search engine when facing a similar patient, and make sure those words appear in your title or abstract.

Writing the Abstract

Many journals require a structured abstract with headings such as Background, Case Presentation, and Conclusion, usually limited to 150–250 words. Write the abstract last, after the rest of the report is finished, so that it accurately reflects what you actually wrote. Every claim in the abstract must also appear in the main text, with the

same numbers and the same wording.

A strong abstract answers four questions in order: Why is this case worth reading? What happened to the patient? What was the outcome? What should readers take away? Do not include references, abbreviations that are not explained, or details that belong only in the main text. Do not overstate the conclusion: write 'this case suggests' rather than 'this proves'.

Keywords

Choose four to six keywords that help databases index your report. Use standard medical terms where possible, include the condition and the unusual feature, and add the phrase 'case report' if the journal allows it. Good keywords make the difference between a report that is found and one that is never read.

Chapter 5: Introduction: Background and Why This Case Matters

The introduction of a case report is short — usually two to four paragraphs — and it has one job: to explain why this case deserves the reader's attention. It is not a textbook chapter on the disease. Readers do not need a full review of everything known about a common condition; they need just enough background to understand what is unusual about your case.

A useful structure is: first, one or two sentences of background that place the condition in context, with a small number of key references. Second, a sentence or two explaining what is already known about the unusual feature, and what remains uncertain. Third, a clear statement of why this case matters — the gap it addresses or the lesson it offers. End the introduction with a sentence that leads naturally into the case presentation, such as: 'We report the case of a patient with...'

How Much Literature to Cite

A case report typically cites 10–20 references, most of them in the introduction and discussion. Cite the most relevant and most recent sources, and prefer original reports and guidelines over secondary summaries. Every reference must be read, not copied from another paper's reference list, and every statement that needs support should have a citation. Do not pad the reference list: reviewers notice, and it weakens the paper.

Common Introduction Mistakes

- Writing a long textbook review instead of a focused background.
- Failing to state clearly what is new or unusual about the case.
- Citing references that do not actually support the statement made.
- Starting with the patient's story — that belongs in the case presentation, not

the introduction.

Chapter 6: Case Presentation: History, Examination, Investigations and Timeline

The case presentation is the heart of the report. It must be complete, accurate, and organised so that a reader who has never met the patient can follow exactly what happened. Write it in chronological order, use the past tense, and give exact numbers, dates, and doses wherever possible. Vague phrases such as 'blood tests were abnormal' are useless; the reader needs to know which tests, what the values were, and what the normal range is.

Patient Information and History

Begin with the essential, non-identifying facts: age, sex, and relevant background (for example, occupation or setting, if it matters clinically). Then describe the presenting complaint in the patient's own terms where helpful, followed by the history of the present illness: when symptoms began, how they developed, what made them better or worse, and what treatments were tried before. Include relevant past medical history, drug history (including over-the-counter medicines, supplements, and traditional remedies), allergies, family history, and social history — but only the details that are relevant to the case. A complete drug list matters when a side effect is suspected; a detailed smoking history matters in a respiratory case and much less in a simple fracture.

Protect anonymity at every step. Do not include the patient's name, initials, date of birth, hospital number, address, or any detail that could identify them. Avoid rare combinations of details (a very rare job in a very small town, for example) that could identify a person even without a name. When in doubt, leave the detail out or make it more general.

Examination and Investigations

Report the examination findings systematically: general appearance and vital signs first, then the relevant systems. Give actual measurements — temperature, pulse, blood pressure, respiratory rate, oxygen saturation — rather than saying 'vital signs were normal'. Record both positive and negative findings: the absence of a sign can be as informative as its presence, especially when it helps to exclude other diagnoses.

For investigations, report the key results with units and reference ranges, and give the dates on which they were done. Include the investigations that led to the diagnosis and those that excluded important alternatives. Do not list every test ever performed; select the ones that matter to the story. Images and test results can be included as figures, but only with the patient's consent and with all identifying

information removed. Label every figure clearly and refer to it in the text.

The Timeline

A timeline is one of the most valuable parts of a case report, and the CARE guidelines specifically recommend it. It shows, at a glance, the order in which symptoms appeared, investigations were done, treatments were given, and the patient improved or worsened. Many readers will study the timeline before reading the text. Keep it simple: date or day, event, and result. The table below shows the format for the fictional worked example in Chapter 10.

Date / Day	Event	Finding / Action
Day 1	Fever and sore throat begin	Temperature 38.6 C; paracetamol started
Day 3	Clinic visit	Throat red; no rash; blood tests sent
Day 5	New symptom appears	Unusual rash on arms; referred to hospital
Day 6	Hospital admission	Investigations confirm diagnosis; treatment started
Day 9	Improvement	Fever settles; rash fading
Day 14	Discharge	Well; follow-up arranged in 2 weeks
Day 28	Follow-up visit	Fully recovered; no recurrence

A timeline also protects you as an author: building it forces you to check the order of events against the medical records, and it often reveals gaps or inconsistencies in your account before a reviewer finds them.

Chapter 7: Treatment, Outcome and Follow-up

Describe exactly what treatment was given: the drug name (generic name, not brand name, unless the brand matters), the dose, the route, the frequency, and the duration. If a procedure was performed, describe it briefly, including who performed it and any complications. If treatment was changed, explain why, and give the dates of each change. A reader who wants to treat a similar patient should be able to reproduce your management from this section alone.

Be honest about what did not work. If the first treatment failed, say so, and say what was tried next. Negative results are valuable: they prevent other clinicians from repeating a mistake. Do not hide adverse effects of treatment, and do not present a treatment as successful if the patient improved for another reason, or if the improvement was only temporary.

Outcome and Follow-up

Report the outcome clearly: did the patient recover fully, partially, or not at all? How long was the patient followed up, and what was their condition at the last follow-up? Follow-up matters because a case that looks like a success after one week may look very different after three months. State the duration of follow-up explicitly — 'the patient remained well at six months' is far more useful than 'the patient is doing well'.

Include the patient's own perspective where possible. How did they experience the illness and the treatment? Were they satisfied with the outcome? The CARE guidelines encourage a short patient-perspective section, and journals increasingly value it. It reminds readers that behind every case is a person, and it often reveals outcomes that matter to patients — such as being able to return to work — that laboratory results do not capture.

Chapter 8: Discussion: What This Case Adds to Medical Knowledge

The discussion is where you explain what the case means. It is usually the longest section after the case presentation, and it is where most case reports succeed or fail. A good discussion does four things: it summarises the key features of the case, it compares the case with what has already been published, it explains what is new and what readers should learn, and it acknowledges the limitations honestly.

Comparing with the Literature

Search the literature systematically and describe what you found: how many similar cases have been reported, how your case is similar, and how it differs. If your case is the first of its kind, say so carefully — 'to the best of our knowledge, this is the first reported case of...' — and explain how thorough your search was. If similar cases exist, explain what your case adds: a different age group, a different setting, a longer follow-up, or a new management lesson.

Explaining the Mechanism

Where possible, offer a plausible explanation for what happened, based on known physiology or pharmacology. Keep speculation clearly labelled as speculation, and do not present a guess as a fact. If the mechanism is unknown, say so: 'the mechanism remains unclear' is an honest and acceptable statement, and it may prompt future research.

Limitations — Be Honest

Every case report has limitations, and stating them strengthens rather than weakens your paper. The main limitation is always the same: a single case cannot prove cause and effect, and it cannot show how often something happens. Other limitations may include a short follow-up, missing investigations, or the fact that the patient received

several treatments at once, so it is unclear which one helped. Reviewers trust authors who recognise these limits. They distrust authors who claim too much.

Take-Home Message

End the discussion with a short, clear take-home message — one or two sentences that a busy reader can remember. For example: 'Clinicians should consider this diagnosis when a patient with fever develops an unexplained rash, even when initial tests are normal.' This is the sentence your readers will carry into their next clinic, so make it practical and precise.

Chapter 9: Patient Consent, Anonymity and Ethics

No case report is worth publishing if it harms the patient it describes. Ethics is not a formality at the end of the process; it is the foundation of the whole report. Before you write a single word for publication, you must have the patient's informed consent, and you must protect their identity throughout.

Informed Consent

Written informed consent must be obtained from the patient (or, for a child or a patient who lacks capacity, from their parent or legal guardian) before submission. The patient must understand what a case report is, that it will be published and available to other people, that it may include photographs or test results, and that they can withdraw consent before publication. Consent should be documented on a form approved by your hospital or journal, and a copy kept in the medical records. Most journals require a statement in the manuscript confirming that consent was obtained, and some ask to see the signed form.

Consent must be freely given. A patient who feels pressured, or who does not understand what publication means, has not truly consented. Take time to explain, answer questions, and give the patient the opportunity to read the draft. If a patient declines, respect that decision immediately and do not write the report. There will be other cases; there is no justification for publishing without consent.

Anonymity

Remove all identifying information: name, initials, date of birth, hospital number, address, workplace, and photographs that show the face or any distinctive feature. Be careful with details that seem harmless on their own but could identify a person when combined — a rare occupation, a small village, and an unusual condition together may be enough. If a photograph is essential, crop it tightly, remove any background that could identify the location, and cover the eyes only if that genuinely protects identity (note that eye bars alone are often not enough, and specific consent for the photograph is still required).

Other Ethical Duties

- Do not fabricate or alter any detail to make the case more interesting. If a fact is uncertain, say so or leave it out.
- Do not publish a case that could embarrass or stigmatise the patient, even with consent, without careful thought and discussion with a senior colleague.
- Declare any conflicts of interest, such as funding or a relationship with a drug company.
- Follow your hospital's and your journal's policies on ethics approval: most case reports do not need formal research ethics approval, but some institutions require notification or a waiver — check before submitting.
- All examples in this report are fictional and clearly labelled as such. No real patient is described anywhere in this document.

Chapter 10: A Complete Worked Example: A Fictional Case Report

The following is a complete, short case report, written as a model. EVERY detail in this example is fictional and invented for teaching purposes. No real patient is described. The case is deliberately ordinary in its condition and unusual in its presentation, which is the most common pattern for a publishable case report: a common condition, seen in an uncommon way.

FICTIONAL CASE — ALL DETAILS INVENTED

Title: Dengue Fever Presenting with Prolonged Hiccups in a Young Adult: A Case Report

Abstract

Background: Dengue fever is a common mosquito-borne illness that usually presents with fever, headache, muscle pain, and rash. Hiccups are a rare and poorly recognised feature of dengue infection. Case presentation: A fictional 24-year-old man developed fever and, on the third day, persistent hiccups lasting more than 48 hours, which led to hospital admission and investigation. Dengue was confirmed by laboratory testing, other causes of hiccups were excluded, and the hiccups resolved as the fever settled. Conclusion: Clinicians in dengue-endemic areas should consider dengue in patients with fever and unexplained prolonged hiccups, and should not overlook this unusual presentation.

Introduction

Dengue fever affects millions of people each year in tropical and subtropical regions. The typical features — high fever, severe headache, pain behind the eyes, muscle and joint pain, and rash — are well known. Less well known are the unusual

presentations that can delay diagnosis. Prolonged hiccups, defined as hiccups lasting more than 48 hours, have many causes, including irritation of the diaphragm, metabolic disturbance, and infection. Very few reports have described hiccups as a presenting feature of dengue fever. We report this fictional case to alert clinicians to this unusual presentation and to show how a careful history and targeted investigation can prevent unnecessary tests and delay.

Case Presentation (Fictional)

A fictional 24-year-old man, a shop assistant, presented to hospital with fever for three days and continuous hiccups for two days. The fever had begun suddenly, with headache and muscle pain. On the second day he had taken paracetamol at home, with little relief. On the third day he developed hiccups that continued day and night, disturbed his sleep, and made eating difficult. He had no cough, breathlessness, chest pain, vomiting, abdominal pain, or rash at presentation. He had no recent travel, no known contact with a person with dengue, and no history of similar illness. His past medical history was unremarkable, he took no regular medicines, and he had no known drug allergies.

On examination, his temperature was 38.8 C, pulse 104 beats per minute, blood pressure 110/70 mmHg, respiratory rate 18 per minute, and oxygen saturation 98 percent on room air. He was alert and uncomfortable because of the hiccups, which occurred every few seconds. Examination of the throat showed mild redness. Chest examination was normal, abdominal examination was normal with no tenderness and no enlarged liver or spleen, and there was no rash and no bleeding. Neurological examination was normal.

Blood tests showed a white cell count of 3.2 x 10^9/L (low), a platelet count of 98 x 10^9/L (low), and a mildly raised liver enzyme level. Blood sugar, kidney function, and electrolytes, including sodium, potassium, and calcium, were normal. A chest X-ray was normal, and an ultrasound of the abdomen showed no abnormality. Dengue NS1 antigen testing was positive, confirming dengue infection. Tests for malaria and typhoid were negative. No other cause for the hiccups was found.

Treatment and Outcome (Fictional)

The patient was admitted for observation and supportive care. He received intravenous fluids, paracetamol for fever, and reassurance about the hiccups. No specific drug was given for the hiccups, because no underlying cause requiring treatment had been found, and the team expected them to resolve as the infection settled. His temperature began to fall on the second day in hospital, and the hiccups became less frequent. By the fourth day in hospital, the hiccups had stopped completely. His platelet count rose to 145 x 10^9/L, and he was discharged on the fifth day with advice to rest, drink plenty of fluids, and return if fever or bleeding occurred. At a follow-up visit two weeks later, he was fully recovered, with no

recurrence of hiccups and a normal platelet count.

Discussion (Fictional Case)

This fictional case illustrates three teaching points. First, dengue fever can present in unusual ways, and a clinician who expects only the textbook features may miss the diagnosis. Second, prolonged hiccups in a febrile patient should prompt a careful search for an underlying infection, including dengue in endemic areas, before extensive neurological or gastrointestinal investigation. Third, supportive care and observation are often sufficient: in this fictional patient, the hiccups resolved without specific treatment as the viral illness settled.

The limitations of this fictional example are clear: it is a single invented case, it cannot prove that dengue caused the hiccups, and the mechanism is not established. In a real report, the author would need to search the literature for similar cases, discuss other possible causes of hiccups in detail, and report a longer follow-up. Nevertheless, as a teaching model, it shows the structure, the level of detail, and the tone that a real case report should aim for: clear, factual, and modest in its claims.

What to Copy from This Model — and What Not To

- Copy the structure: title, abstract, introduction, case, treatment, outcome, discussion, and a clear take-home message.
- Copy the level of detail: exact numbers, dates, doses, and test results with units.
- Copy the honest tone: what was found, what was not found, and what remains uncertain.
- Do NOT copy the patient or the facts: this case is invented. Your report must describe a real patient, with real records, and with real consent.

Chapter 11: Common Mistakes and a Final Checklist

Most case reports are rejected for preventable reasons. The mistakes below appear again and again in reviewers' comments. Avoiding them will not guarantee acceptance, but it will give your report a fair chance.

Common Mistakes

1. Choosing a case with no clear lesson. If you cannot state the lesson in one sentence, the case is probably not ready.
2. Writing too much background and too little case. The case presentation is the heart of the report; the introduction should be short.
3. Giving vague results. 'Blood tests were abnormal' tells the reader nothing. Give the test, the value, the unit, and the normal range.
4. Omitting the timeline, the follow-up duration, or the exact treatment doses.

5. Overclaiming. One patient cannot prove that a treatment works. Use 'suggests', 'may', and 'appears to', not 'proves'.

6. Forgetting consent or anonymity. A report without documented consent will be rejected, and may cause serious harm to the patient.

7. Copying text from other papers. All writing must be your own, and every source must be cited properly.

8. Ignoring the journal's instructions. Every journal has specific requirements for word count, structure, references, and figures. Read them before you submit, not after rejection.

9. Submitting without a senior colleague reading the draft. A fresh pair of eyes will find errors, gaps, and unclear sentences that you can no longer see.

10. Poor language and careless errors. Spelling mistakes, inconsistent numbers, and unclear sentences signal carelessness and undermine trust in the clinical facts.

Final Checklist — Before You Submit

Work through this checklist with a printed copy of your manuscript and the journal's author instructions beside you. Tick each item only when you have actually checked it, not when you think it is probably fine.

☐ The case has a clear, one-sentence lesson, stated in the abstract and the discussion.

☐ The title is specific, includes the key feature, and includes the words 'case report'.

☐ The abstract is accurate, within the word limit, and matches the main text.

☐ The introduction is short and explains why this case matters.

☐ The case presentation is complete: history, examination, investigations with values and units, and a timeline.

☐ No identifying information about the patient appears anywhere, including in figures.

☐ Treatment is described fully: drug names, doses, routes, frequency, and duration.

☐ The outcome and the duration of follow-up are stated clearly.

☐ The discussion compares the case with the literature and states the limitations honestly.

☐ Written informed consent has been obtained, documented, and stated in the manuscript.

☐ The patient perspective is included where possible and appropriate.

☐ All references have been read, are accurate, and are formatted as the journal requires.

☐ The CARE checklist has been completed and is ready to submit if the journal asks for it.

☐ A senior colleague has read the full draft and their comments have been

addressed.

☐ The manuscript follows the journal's word count, structure, and formatting instructions.

☐ All numbers, dates, and doses have been checked against the original medical records.

Chapter 12: Conclusion

Writing a medical case report is an act of careful observation and honest communication. It begins at the bedside, with noticing something unusual and asking whether it might teach others something useful. It continues with thorough record-keeping, a careful search of the literature, and respect for the patient whose story you are telling. And it ends with clear, structured writing that gives readers the facts they need and the lesson they can use.

Case reports will never replace large clinical trials, and they should never pretend to. Their value is different: they notice what trials miss, they teach clinical reasoning, and they give new clinicians a first, achievable step into medical writing. A single well-written case report can change how a reader approaches the next similar patient — and that, for a short document describing one person, is a remarkable achievement.

If you remember only three things from this report, let them be these. First, choose a case with a clear lesson and verify every fact against the records. Second, follow the CARE structure, so that readers can find what they need and trust what they find. Third, never publish without genuine informed consent and complete anonymity. Do those three things, write clearly and honestly, and your case report will deserve to be read.

Suggested Reading

The following are general, well-known resources for anyone learning to write case reports. They are described here in general terms; consult the current editions and the journals' own author instructions for the latest requirements.

- The CARE guidelines for case reports (CAse REport guidelines): the international reporting standard for case reports, including a checklist of the items that should be included in every report. The guidelines and checklist are freely available online from the CARE group and are recommended by many medical journals.
- The EQUATOR Network: a general resource that collects reporting guidelines for many types of health research, including case reports, and explains how to use them.
- Author instructions of major medical journals that publish case reports: these give practical requirements for word count, structure, abstracts, references, figures, and consent statements. Always read the instructions of the journal you intend to submit to before writing.
- Standard textbooks of medical writing and scientific writing: general guides to clear academic English, structuring an article, citing sources, and responding to reviewers' comments.
- Guidance on patient consent and confidentiality published by medical councils and ethics bodies: general principles on informed consent, anonymity, and the responsible use of patient information in publications.

A final note on references: a case report should cite the sources that were actually read and that directly support the statements made. Do not copy reference lists from other papers, and do not invent citations. If you have not read a source, do not cite it.

— End of Report —

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